جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : سافيناز محمدى حسين نوار

المحافظة : القليوبيه **الحي / المدينة :** بنها **العنوان :** عماره 4 م أ مدينه الجامعات

رقم البطاقة : 25902060102161 **رقم التليفون :** 01289307222 **تاريخ تسجيل الإستمارة :** 17-10-2026

العمر(تاريخ الميلاد) : 06-02-1959 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي
التوقيع:(----------)

المقر بما فيه
التوقيع:(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مسح ذري علي العظام [12966] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: سافيناز محمدى حسين نوار
National ID: 25902060102161

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

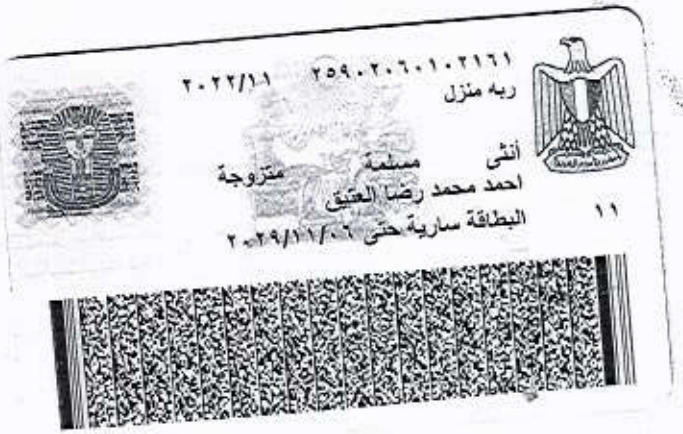
تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Bone scintigraphy

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

03/09/2026

MDT FORM

Name:	سفيناز محمدى حسين لولو	**MRN:**	9159
Age:	67 Y	**ID:**	25902060102161
Menopausal Sta		**Phone:**	01289307222
NRS SCORE:		**Marital Status:**	
BMI:		**Family History:**	
CO Morbidity:		**Reason Of MDT:**	

Clinical Data:

BX:

ER: 8/8 **PR:** 8/8 **HER2:** 2+ sish n **KI67:**

Sonomammography:

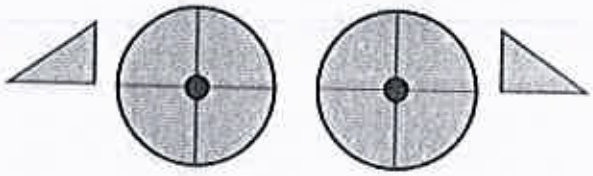
Mets Workup:
met breast to bone bilat pulm

History:

Recommendations:
for AI ZOMETA

MDT Outcome & Plan
for AI ZOMETA

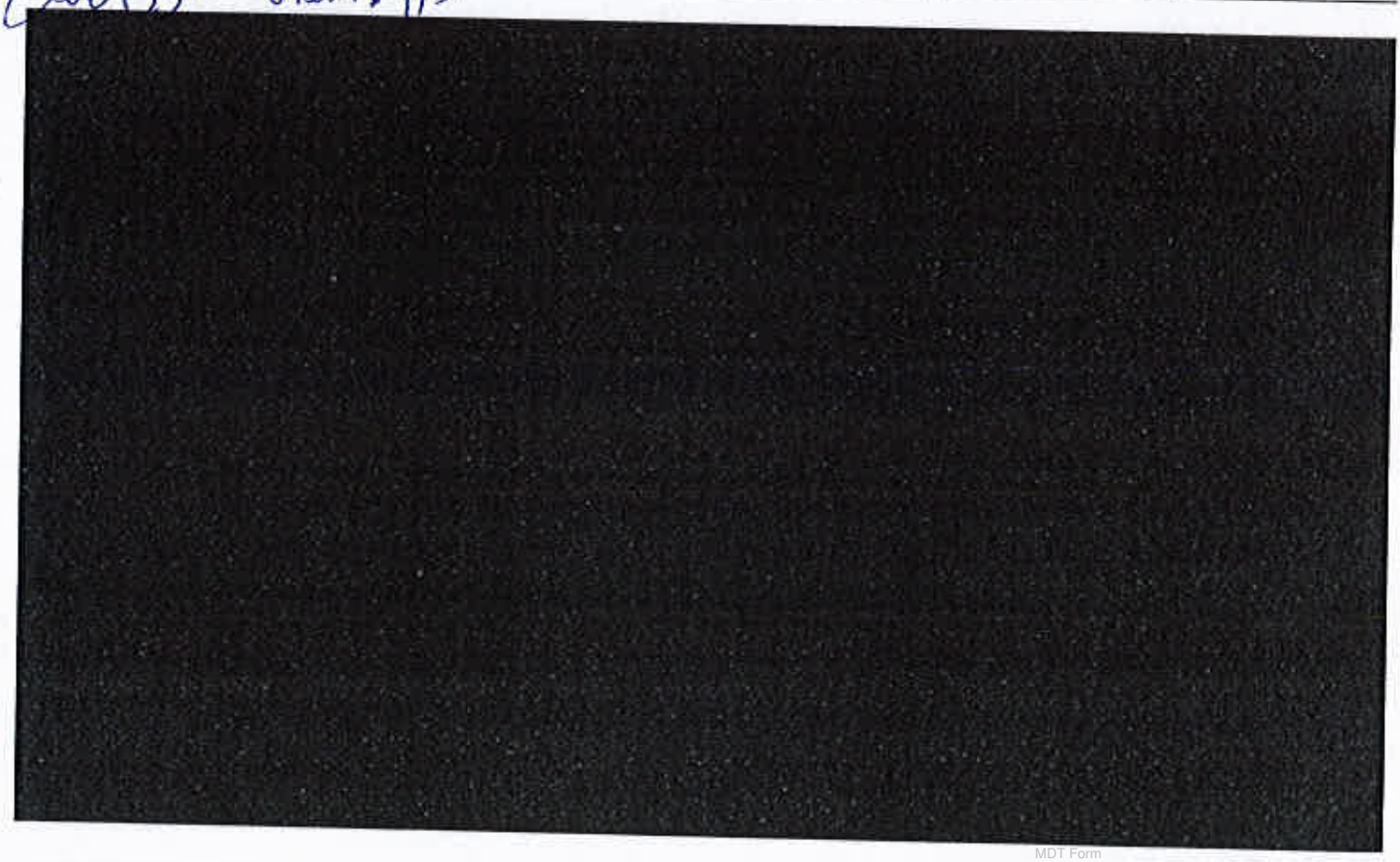
R L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ ترمين مصطفى	د/ خالد السعدي				



MDT Form
[[DOCTYPE:MDT_FORM]]

Name: سافيناز محمدى حسين نوار **MR. :** 9159	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient presents with a history of breast cancer with metastatic disease to bone and lung. Initial pathology demonstrated ER 8/8, PR 8/8, Ki-67 60%, and HER2 equivocal; SISH testing was pending at the time of the first note. The patient was scheduled for multidisciplinary team review following the SISH result.

On the subsequent visit, the patient was noted to have post-mastectomy left breast cancer, classified as T4b N3 M1 with metastases to lung and bone. Pathology again showed ER 8/8, PR 8/8, HER2 2+ with SISH negative for amplification. The patient was evaluated for initiation of aromatase inhibitor therapy (FEMARA) and zoledronic acid (ZOMETA) and was scheduled for an MDT meeting on 9 March 2026. Physical examination revealed a 6 × 7 cm left breast mass with associated edema and a 2 × 2 cm left axillary lymph node. The patient also reported left lower limb weakness and required a dental examination.

In the most recent encounter, the patient completed a right hip fracture repair on 11 April 2026. The patient was started on FEMARA for a planned 3-month course and monthly ZOMETA following laboratory evaluation.

تحريرا في ,

التاريخ :	**19-09-2026**	**الوقت :**	

الطبيب المعالج : ندى حسن صلاح كامل

يعتمد ,

GUSTAVE ROUSSY
INTERNATIONAL CANCER CAMPUS

Patient Name	: الأستاذه / سافيناز محمدي حسين	Request No.	: 13005074
Patient No.	: 25902060102161	Request Date	: 16/02/2026
Path No.	: 461/26	Verification Date	: 22/02/2026
Sex	: Female	Financial Category	: جوستاف روسي - اقتصادى
Age	: 67 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass (BIRADs 5), diagnosed by (H&E) as invasive duct carcinoma in a request No (13005074).
Nature of Specimen: Tru-cut biopsy.

Technique:
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- P63 clone (4A4), ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:
* IPT (Positive control showed appropriate reactivity):
- P63: Negative.
- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 80% of tumor cells (score 8/8).
- Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).
- Ki67%: 50%.

Conclusion:
Left breast mass, Tru-cut biopsy:
IPT:
- P63: Negative.
- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Equivocal (score 2+).
* Case for further (SISH).
- Ki67%: 50%.

* Scoring system: "ER, PR"

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name	الأستاذه / سافيناز محمدي حسين	Request No.	13005074
Patient No.	25902060102161	Request Date	16/02/2026
Path No.	461/26	Verification Date	22/02/2026
Sex	Female	Financial Category	جوستاف روسي - اقتصادى
Age	67 Y	Department	Out Patient
Referreing Doc.		Registered by	Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0 (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Dr. Walaa Ibrahem Ghoneim.
Dr. Heba El-Shafeey.

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

USTAVE OUSSY

Patient Name	: الأستاذه / سافيناز محمدي حسين	Request No.	: 13005074
Patient No.	: 25902060102161	Request Date	: 16/02/2026
Path No.	: 461/26	Verification Date	: 21/02/2026
Sex	: Female	Financial Category	: جوستاف رومي - اقتصادى
Age	: 67 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical data
Left breast mass.
Radiology results:
-Highly suspicious looking mass(an irregular shaped mass with speculated margin and multiple foci grouped microcalcifications inside is seen in the lower upper quadrant) associated with an adjacent area of distrortion BIRADs 5.

Nature of specimen
Tru cut biopsy, collected on 3:00 pm, cold ischemia time is 10 sconds.

Gross examination
Multiple (4) grayish white tissue cores raning in size from 0.8 up to 1.2 cm, totally submitted.

Microscopic examination
* Histological type: Invasive duct carcinoma.
* Grade (differentiation +nuclear pleomorphism+ mitosis): (3+3+2) Grade 2
* DCIS: Absent.
* Angioinvasion:Absent.
*Perineural invasion:Absent.

Diagnosis
Left breast mass, Tru cut biopsy:
Invasive duct carcinoma, Grade 2.
For further IHC (ER, PR, HER2, Ki67)

Signature
Prof. Dr. Gamal Abdel Aty
Dr.Menan Hosam

Verified by :
Dr.Heba Hasan

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

- Unremarkable posterior fossa structures.

B. Neck:

- **FDG avid left deep cervical LN is noted, measuring about 1.6x1.3 cm with SUVmax~6.1 likely metastatic.**
- No FDG avid thyroid nodules.
- Normal appearance and physiologic FDG uptake of the nasopharynx, oropharynx and laryngeal air spaces.
- Normal appearance and physiologic FDG uptake of submandibular and parotid salivary glands.

C. Chest:

- **The left breast showing mild diffuse skin thickening with FDG soft tissue lesions with lobulated outline at its UOQ reaching the skin, measuring roughly about 8.1x8.5x7.2 cm along maximum axial & CC dimensions with SUVmax~20.8.**
- **Few FDG avid left axillary & sub-pectoral LNs are noted, with largest seen measuring about 1.7x1.3 cm with SUVmax~5.4.**
- No FDG avid right breast lesions or right axillary LNs.
- **FDG avid few bilateral scattered pulmonary nodules "likely metastatic" are noted, with largest seen at right upper lung lobe measuring about 2.3x1.7 cm with SIVmax~12.3.**
- **FDG avid mildly enlarged pre-vascular LN is noted "likely metastatic", measuring about 1.9x1.4 cm with SUVmax~20.8**
- No evidence of pleural effusion, pleural thickening or masses seen.
- No pericardial effusion, thickening or masses detected.
- Normal physiologic FDG uptake of the heart.

D. Abdomen and Pelvis:

- **No hyper-metabolic abdominal or pelvic lymphadenopathy.**
- Enlarged liver with no FDG avid focal lesions.

الممسوحة ضوئيا بـ CamScanner
PET Scan
[[DOCTYPE:PET_SCAN]]

مؤسسة مستشفى سرطان الأطفال - مصر
Children's Cancer Hospital Foundation - Egypt
57357

Nuclear Medicinine Dept.
PET/CT Unit

9109

Name: Mrs. Safinaz Mohammady Hussein Date: 18/02/2026
ID: 30207191

Whole Body 18F-FDG PET/CT

History and clinical findings:
67-year-old female patient with newly diagnosed left breast IDC.

Reason for the study: Initial PET/ CT assessment.

Technique:
Radio-pharmaceutical: F-18-FDG, MBq intra-venous infusion.
Imaging sequence:

* *Approximately 60 minutes following radiopharmaceutical administration, PET-CT was acquired from mid-thigh to the skull vault with the arms extended above the head.*
* *Qualitative and semi-quantitative assessment of the PET images was done.*
* *Diagnostic CT was acquired in addition to the purpose of anatomic localization and attenuation correction.*
* ***IV contrast was administered.***
* *Data was processed and displayed according to departmental protocol.*

FINDINGS:

A. Brain:

- **No gross hyper-metabolic brain SOLs.**
- No gross focal cerebral or cerebellar area of abnormal density seen.
- No gross extra-axial or intra-cerebral areas of fresh blood densities.
- No midline shift.

1 Seket Al-Emam st, El Sayda Zainab, Cairo
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Fax: (202) 25 35 1745
E-mail: info@57357.org

١ شارع سكة الإمام - السيدة زينب - القاهرة
ص . ب : ١ الدواوين - مصر
الهاتف : (202) 19057
فاكس : (202) 25 35 1745
57357Egypt

CamScanner الممسوحة ضوئيا بـ
PET Scan
[[DOCTYPE:PET_SCAN]]

- Physiologic diffuse FDG uptake is seen over average size spleen with no FDG avid focal lesions seen.
- Normal appearance of both kidneys with no back-pressure changes. Physiologic FDG excretion is seen in the kidneys and bladder.
- ***Prominent left adrenal gland.*** Yet no active adrenal nodules.
- Normal appearance of the pancreas, aorta & I.V.C with no FDG avid lesions detected.
- *Mesenteric panniculitis noted.*
- No FDG avid uterine or adnexal lesions.
- No evidence of ascites or abdominal collections noted.

E. **Musculoskeletal:**

- **FDG avid multiple lytic & sclerotic metastatic osseous lesions are seen involving axial & appendicular skeleton dominant at the spine and pelvic bones showing variable grades of FDG activity of SUVmax~10.9.**
- *Diffuse spondylo-degenerative changes are noted.*
- *Ist degree degenerative spondylolisthesis of L4 over L5.*
- *Bilateral sacroiliitis, more on the left.*

الممسوحة ضوئيا بـ CamScanner
PET Scan
[[DOCTYPE:PET_SCAN]]

Conclusion:

Initial FDG PET/CT study revealed:

- Left breast FDG soft tissue lesions with lobulated outline at its UOQ reaching the skin, (site of primary neoplasm).
- Few FDG avid left lower cervical, mediastinal prevascular, left axillary & sub-pectoral LNs, (metastatic deposits).
- FDG avid few bilateral scattered pulmonary nodules "likely metastatic".
- Wide spread FDG avid metastatic osseous deposits.

Thanks for referral;

Dr. Rasha Anwar, MD.
Consultant of Radio-diagnosis.

Dr Esraa El-Kholy, MD
Consultant of Nuclear Medicine, CCHE
Professor of Nuclear Medicine, NCI
Cairo University

الممسوحة ضوئيا بـ CamScanner
PET Scan
[[DOCTYPE:PET_SCAN]]

DMS: 9/59

Name : SABINAZ MOHMADYHUSSEIN Date : 16-02-2026 01:10
Mach : Portable Medical No. : GRI-160226010852PM
Ref.Physician Prof : Ins.Company :

MATERAL MAMMOGRAM AND BREAST ULTRASONOGRAPHY REVEALED:

Indication: Palpable left breast mass.

- Breast density: Predominantly fatty breast parenchyma...Matching ACR a.
- An irregular shaped mass with speculated margin and multiple foci grouped micro-calcifications inside is seen in the lower upper quadrant of the left breast, associated with adjacent area of architectural distortion retracting the nipple
- No detected suspicious mass density, architectural distortion or pathological micro-calcification in the right breast.

By complementary breast ultrasound:

- Homogenous (predominantly fatty) echo-texture of both breasts.
- An irregular shaped non circumscribed margin hypoechoic mass is seen in the left breast at 1-2 o'clock position. it measures (8.5 x 6.8 x 7.7 cm) ,reaching the overlying skin , as well as internal vascularity detected within
- *No detected suspicious solid or cystic masses in the right breast.*
- Few indeterminate left axillary lymph nodes, largest measures (2.4 x 1.9 cm) with cortical thickness of 4.3 mm
- *Pathological left infraclavicular lymph node with globular shape and lost cortical hilum measuring 15 x 12 mm.*
- *Right indeterminate axillary lymph node measuring 20 x 8 mm with cortical thickness of 3.5 mm*

CONCLUSION:

- *Highly suspicious looking left breast mass associated with an adjacent small area of distortion BI-RADS 5 category.*

Dr farah elshobaky

Mammogram
[[DOCTYPE:MAMMOGRAM]]



٢٥٩٠٢٠٦٠١٠٢١٦١ ٢٠٢٢/١١
ربه منزل

انثى مسلمة متزوجة
احمد محمد رضا العتيق
١١ البطاقة سارية حتى ٢٠٢٩/١١/٠٦



National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]